Order No:485-24420

HOME HEALTH CERTIFICATION AND PLAN OF CARE

1. Patient's HI Claim No.	2. Start Of Care Date	3. Certification Period	4. Medical Record No.	5. Provider No.
918684072	01/26/2026	01/26/2026 - 03/26/2026	9128	1548211717

6. Patient's Name and Address
Wehman, Geraldine (9128)
3010 Airline Rd
Corpus Christi, TX 78414
Phone: (361) 947-2407

7. Provider's Name, Address and Telephone Number
PAM Health at Home
625 Main Street, Ste 110 Buda, TX
Buda, TX 78610-3273
Phone: (512) 295-7000, Fax: (512) 295-7070

8. Date of Birth: 05/22/1942 **9. Sex:** ☐ M ☒ F

24. Physician's Name and Address
JOANN WEBB MD
7121 S PADRE ISLAND DR STE 300
Corpus Christi, TX 78412,NPI: 1073119970, License#: 1021049
,Phone: (361) 696-6200, Fax: (361) 696-6060

26. I certify this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy, or continues to need occupational therapy. This patient is under my care, and I have authorized the services on this plan of care, and will periodically review the plan. I further certify this patient had a face-to-face encounter that was performed by a physician or Medicare allowed non-physician practitioner that was related to the primary reason the patient requires home health services.

27. Attending Physician's Signature and Date Signed:

2/25/26

28. Anyone who misrepresents,falsifies, or conceals essential information required for payment of Federal funds may be subject to fine, imprisonment, or civil penality under applicable Federal laws.

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10. Medications: Dose/Frequency/Route (N)ew (C)hanged

Medication	Dose	Frequency	Route	Status
Acetaminophen [Amount Of/ Dosage Form: 2]	325MG	3 times a day	ORAL	LS
Amlodipine 5 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	5MG	daily	oral	LS
Calcium 600 mg (as carbonate)-vitamin D3 5 mcg (200 unit) tablet [Amount Of/ Dosage Form: 1 Tablet]	600-10MG	daily	oral	LS
Calcium carb, citrate-vit D3 [Amount Of/ Dosage Form: 1]	1500 OSCAL 600CA	daily	ORAL	LS
Celecoxib 200 mg capsule [Amount Of/ Dosage Form: 1 Capsule]	200MG	daily	oral	LS
Cetirizine [Amount Of/ Dosage Form: 1]	10MG	daily	ORAL	LS
Cholecalciferol (vitamin D3) [Amount Of/ Dosage Form: 1]	25MCG	daily	ORAL	LS
Citalopram 20 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	20MG	daily	oral	LS
Cyclobenzaprine 5 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	5MG	2 times a day AS NEEDED FOR BACK PAIN	oral	LS - PRN/As Needed
Divalproex [Amount Of/ Dosage Form: 1]	125MG	1	ORAL	LS
Lactulose [Amount Of/ Dosage Form: 15]	10GM/15ML	3 times a day AS NEEDED FOR CONSTIPATION	ORAL	LS - PRN/As Needed
Memantine 10 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	10MG	daily	oral	LS
Metoprolol tartrate 25 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	25MG	2 times a day	oral	LS
Quetiapine 25 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	25MG	PM	oral	LS
Rivastigmine 9.5 mg/24 hour transdermal patch [Amount Of/ Dosage Form: 1 Patch]	9.5MG	daily	transdermal	LS
Rosuvastatin 5 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	5MG	EVENING	oral	LS
Seroquel 50 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	50MG	at bedtime	oral	LS
Thera-Tabs tablet [Amount Of/ Dosage Form: 1 Tablet]	1	daily	oral	LS

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11. ICD-9 / 10 CM Principal Diagnosis

Code	Code Description	Date	O/E
S22.080S	Wedge compression fracture of T11-T12 vertebra, sequela		

12. ICD-9 / 10 CM Surgical Procedure

Code	Code Description	Date	O/E

13. ICD-9 / 10 CM Other Pertinent Diagnosis

Code	Code Description	Date	O/E
G89.29	Other chronic pain		
G30.1	Alzheimer's disease with late onset		
F02.B0	Dementia in other diseases classified elsewhere, moderate, without behavioral disturbance, psychotic disturbance, mood disturbance, and anxiety		
I10	Essential (primary) hypertension		
M47.16	Other spondylosis with myelopathy, lumbar region		
S52.532K	Colles' fracture of left radius, subsequent encounter for closed fracture with nonunion		
W19.XXXS	Unspecified fall, sequela		
Z91.81	History of falling		
Z79.1	Long term (current) use of non-steroidal anti-inflammatories (NSAID)		
Z79.899	Other long term (current) drug therapy		

14. DME and Supplies:
DME: Tub/Shower Bench
MASK, ALCOHOL WIPES, NS GLOVES, PROBE COVERS

15. Safety Measures:
911 for Emergency, Assistance with IADLs, Emergency Plan Developed, Fall Precautions, Infection Control Standard Measures and Precautions, Keep Pathways Clear and Safe, Medication Safety / Storage, Safe Transfers, Safety in ADLs, Skin Care, Slow Position Change, Support During Transfer and Ambulation, Universal Precautions

16. Nutritional Req.
No Added Salt,
HEART HEALTHY

Medicare#:

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17. Allergies:
No Known Allergies

18.A. Functional Limitations
☐ 1- Amputation ☐ 4- Hearing ☒ 7- Ambulation
☐ 2- Bowel/Bladder (Incontinence) ☐ 5- Paralysis ☐ 8- Speech
☐ 3- Contracture ☒ 6- Endurance ☐ 9 - Legally Blind
☐ A- Dyspnea with minimal exertion
☐ B- Other (Specify):

18.B. Activities Permitted
☐ 1- Complete Bedrest ☒ 5- Exercises Prescribed ☐ 9- Cane
☐ 2- Bedrest BRP ☐ 6- Partial Weight Bearing ☐ A- Wheelchair
☐ 3- Up as Tolerated ☐ 7- Independent at Home ☐ B- Walker
☐ 4- Transfer Bed/Chair ☐ 8- Crutches ☒ C- No Restrictions
☒ D- Other (Specify):SBA

19. Mental Status
☒ 1 - Oriented ☐ 2 - Comatose ☒ 3 - Forgetful ☐ 4 - Depressed ☐ 5 - Disoriented ☐ 6 - Lethargic ☒ 7 - Agitated
☐ 8 - Other(Specify):

20. Prognosis: ☐ 1- Poor ☐ 2- Guarded ☒ 3- Fair ☐ 4- Good ☐ 5- Excellent ☐ 6- Other (Specify):

21. Treatments and Orders for Disciplines:
Frequency
SN Frequency: 1M2 Starting January 2026,PT Frequency: 2W3 Starting Week of 01/26/2026

Vitals Normal Range
BP Systolic: 90 - 160
BP Diastolic: 56 - 100
Pulse: 56 - 100
Respiratory: 12 - 28
Temperature: 96.5 - 100.5
O2 Saturation: 90 - 100

Vitals Physician Alert Range
Temperature: < 96.5 or > 100.5
Pulse: < 56 or > 100
Respiratory: < 12 or > 28
BP Systolic: < 90 or > 160
BP Diastolic: < 56 or > 100
O2 Saturation: < 90
Weight: < 101 or > 111
Pain: > 7

SN
BEST PRACTICE FALL PREVENTION:
- Clinician will perform fall assessment and instruct patient /caregiver on fall prevention.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION:

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- Clinician to perform drug regimen review/assessment to assess for potential clinically significant medications issues.
- Clinician to report potentially clinically significant medications issues to physician within 1 calendar day.
- Clinician to assess/instruct on high risk and other medications to include effects, potential adverse effects, interactions and duplicate therapy, dose route and frequency.

BEST PRACTICE MONITOR AND MITIGATE PAIN:

- Clinician will instruct patient /caregiver on methods to mitigate pain to include medications and non-pharmacological interventions to decrease pain.

BEST PRACTICE PRESSURE ULCER PREVENTION:

- Clinician will perform teaching and training to patient/caregiver for pressure ulcer prevention.

HOSPITALIZATION RISK FACTORS:

- Clinician will instruct the patient and/or caregivers on the following unstable disease processes that may lead to unplanned hospitalizations: COMPRESSION FRACTURE OF T11-T12 VERTEBRA, CHRONIC PAIN, ALZHEIMER'S DISEASE, DEMENTIA, HTN
- Clinician will instruct patient/caregiver on ways to prevent unintentional weight loss and poor nutrition (high calorie foods, recording (wt/diet), etc
- Clinician will instruct patient and/or caregivers on polypharmacy, duplicate medications, adverse effects, et and compliance/understanding of medication regimen.
- Clinician will assess and teach patient and/or caregivers regarding declines in mental, emotional, or behavior status and when to report to the MD.
- Clinician to instruct patient and/or caregivers on ways to avoid exhaustion and fatigue to include adequate sleep, nutrition, exercise, etc.
- Clinician will instruct patient and/or caregivers on fall prevention to include clear pathways, adequate lighting, assistive devices, slow change in position, etc.

CARDIOVASCULAR - HTN:

- SN to instruct pt/cg on ways to manage HTN such as taking medications as prescribed, increasing physical activity, increasing foods/fluids high in potassium and calcium, managing stress, and avoiding high sodium foods, limit alcohol and monitor blood pressures at home.
- SN to instruct patient/caregiver that uncontrolled HTN can lead to heart attack, stroke, heart failure, kidney complications, vision loss, metabolic syndrome and vascular dementia. SN to instruct pt/cg that HTN is known as the silent killer and symptoms may not be obvious.
- SN to instruct the patient/caregiver on the following symptoms could be signs of a heart attack: chest discomfort, discomfort in one or both arms, back, neck, jaw, stomach, shortness of breath, cold sweat, nausea, or dizziness. Instruct patient/caregiver on signs and symptoms that necessitate calling 911.
- SN to instruct patient/caregiver on Heart Healthy diet.

PT

PHYSICAL THERAPY:

- Establish Home Exercise Program and Activity Program
- Home safety evaluation and fall prevention/teach fall risk reduction strategies
- Therapeutic Exercises: PROM/AAROM/AROM

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- Therapeutic Exercises: Balance, coordination, and/or agility training
- Therapeutic Exercises: Back Pain relieving stretches and activiites,Body Mechanics, postural training and positioning
- Gait Training: c balance training

Assessment Summary

FOCUS OF CARE: COMPRESSION FRACTURE OF T12 VERTEBRA
CLINICAL SUMMARY: PATIENT HAS BEEN LIVING IN ALF VIERA. PATIENT HAS HX OF T12 COMPRESSION FRACTURE AND COMPLAING OF PAIN DAILY PATIENT IS AMBULATORY WITH NO AD. LAST FALL WAS APROX 8 MONTHS AGO. PATIENT IS ALERT AND ORIENTED X2 TO PERSON AND PLACE. PATIENTS SON IS PRESENT. PATIENT IS UNABLE TO GIVE ANY MEDICAL HISTORY. PATIENTS MOOD CHANGED FROM ANGRY TO HAPPY SEVERAL TIMES DURING SN VISIT. ALL SKIN IS INTACT. PATIENT GOES TO DINING ROOM FOR ALL MEALS AND PRESENTS WELL NOUISHED. ALL MEDICATIONS ARE ADMINISTERED BY FACILITY MEDICATION AIDE. PATIENT WILL BENEFIT FROM PHYSICAL THERAPY EVALUATION TO DEVELOP HEP TO IMPROVE STRENGTH, ENDURANCE, FALL PREVENTION, AND PAIN MANAGEMENT.
MEDICAL HISTORY:T12 COMPRESSION FRACTURE, , CLOSED FRACTURE OF COLLES FRACTURE OF LEFT RADIUS WITH NON UNION, HTN, ALZHEIMERS DEMENTIA MODERATE TO LATE ONSET, HX OF FALLS, RESIDES IN RESIDENTAL INSTITUTION, ANXIETY, HX OF FRACTURES HIP 2 YRS AGO THAT DID NOT NEED SURGERY. HX OF FACTURED TAILBONE APROX ONE YEAR AGO.
APPTS:PATIENT HAS BEEN REFERRED TO ORTHOPEDIC SPECIALISTS FOR EVAL AND POSSIBLE PAIN MANAGEMENT INTERVENTION.

Frequency may be completed with both face to face visits and/or telehealth (if appropriate to meet patient treatment needs).
All qualified disciplines may do non-billable visits to supervise staff and collect oasis data as needed.
Within 5 days of the end of episode, a qualified professional may have a PRN visit to complete a comprehensive/OASIS assessment to establish the need for continuing home health services or discharge.
May accept orders from all physicians caring for the patient.
Any qualified disciplines may perform visit for resumption of care when patient discharged home from inpatient facility.
May resume all care prior to the hospitalization.
PAM Health at Home may discharge patient if remains in facility when certification period expires.
All skilled disciplines may check oxygen saturation prn sob.

I certify that the patient is confined to his/her home and needs intermittent skilled nursing, physical therapy, occupational therapy and/or speech therapy for the next 60 days.
I attest to the following:
1. GERALDINE WEHMAN is under my care.
2. I have received the following patient records from PAM Health at Home:
a. Patient Summary Form
b. Hospital/LTC stay information (if applicable)
c. Home Health Plan of Care
d. And the signature below is my signature
e. These patient records were considered in advance of my certification of the patient's eligibility for home health services under Medicare.
I certify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy for the next 60 days. This patient is under my care, and I have

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authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter has occurred BY JASON LIU, MD on 01.05.2026 and it is related to the primary reason the patient requires home health services. PAM HEALTH AT HOME, HOME HEALTH AGENCY, MAY PERFORM SOC ON 01.26.2026.

Risk of Hospitalization

Risks: History of falls (2 or more falls - or any fall with an injury - in the past 12 months); Decline in mental, emotional, or behavioral status in the past 3 months; Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months; Currently taking 5 or more medications; Currently reports exhaustion;
Interventions: Referrals - SN,PT; Patient/Family Education; Fall Prevention Program; Individualized Patient Emergency Care Plan; Care Coordination (Physicians, Hospitals, Nursing homes...)
Risk Factors: Help with managing Medications needed (M2020); More than 2 Secondary Diagnoses (M1022 & M1024); Confusion (M1710); Dyspnea (M1400) ; ADL Assistance Needed (M2102 & M2110)

Psychosocial Assessment

No problem found

Cognitive Status

Cognitive status: Memory deficit: failure to recognize familiar persons/places, inability to recall events of past 24 hours, significant memory loss so that supervision is required; Impaired decision-making: failure to perform usual ADLs or IADLs, inability to appropriately stop activities, jeopardizes safety through actions; Verbal disruption: yelling, threatening, excessive profanity, sexual references, etc

Advance Directive

Copies Requested
Patient's Advance Directive - Advanced Care plan;
Have an Advanced Care Plan and it is documented in the medical record: No - No discussion;
Have a Surrogate Decision Maker and information documented in the medical record: No - Unable to provide;

Homebound Status

Patient Homebound Reasons
- Is the Patient Homebound: Yes
- Homebound Reasons: Need assistance for all activities; Residual weakness; Confusion, Unable to go out from home alone; Unable to safely leave home unassisted;
- Patient Chronic and Stable: No
- Patient leaving the home require Considerable and Taxing effort : Yes
- Comments: PATIENT IS CONFUSED , UNSTEADY GAIT, EASY FATIGUE

22. Rehabilitation Potential/Discharge Plans/Goals :

Rehab Potential

Fair to achieve stated goals with skilled interventions and patient's compliance with plan of care.

Discharge Plan

Discharge when medical condition is stable and patient is no longer in need of skilled services

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Goals

SN

BEST PRACTICE FALL PREVENTION: - LTG
- LTG-Patient/caregiver will verbalize understanding of interventions to prevent falls by end of episode.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION: - LTG
- LTG-Patient /caregiver will be able to verbalize/demonstrate correct dose, route, and frequency of all medications and will be able to recall adverse effects to report to MD by end of episode.

BEST PRACTICE MONITOR AND MITIGATE PAIN: - LTG
- LTG-Patient/caregiver will demonstrate decreased pain with ambulation as evidenced by increased activity tolerance by end of episode.

BEST PRACTICE PRESSURE ULCER PREVENTION: - LTG
- LTG-Patient/Caregiver will demonstrate/verbalize understanding of methods to prevent pressure ulcers by end of episode.

HOSPITALIZATION RISK FACTORS: - LTG
- Patient and/or caregivers will verbalize understanding of ways to mitigate risk of return to the hospital and unplanned ER use.

CARDIOVASCULAR - HTN: - LTG
- Patient/caregiver will be able to verbalize ways to manage HTN by end of episode.

PT

HEP: - STG
- STG-Patient and/or caregiver will complete Home Exercise Program that include back pain relieving activites/stretches, within __2__ weeks.

MOBILITY/GAIT: - LTG
- LTG-Patient will demonstrate improved mobility as evidenced by a categorical change with a decreased TUG time from 60__ seconds to _20__ seconds, in order to reduce fall risk and promote safe functional mobility, within _4__ weeks. or at end of episode.
- LTG-Patient will demonstrate improved mobility as evidenced by the increased ability to ambulate without AD and s instability for 150 feet or more, in order to ambulate safely to dinning room and back to Apt, within _4__ weeks. or at end of episode.
- LTG-The pt will report decreased back pain from 3/10, to 1/10 or better, within __4_weeks. or at end of episode.

23. Clinician's Signature and Date of Verbal SOC Where Applicable: (Electronically Signed)
Verbal SOC Date: 01/26/2026
Signed By: Monterd, Jamie - RN on 01/26/2026 10:36 AM, Cosigned by Strait, Donna - RN

25. Date HHA Received Signed POT:

Medicare#: